

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<think >

** Finding reasoning paths : **

- 1 . Short attention span -> Executive functioning deficit -> Hyper activity .
- 2 . Distractibility -> Attention deficit -> ADHD .
- 3 . Forgetfulness -> Working memory deficit -> ADHD .
- 4 . Hyperactivity -> Inattention -> ADHD .

** Reasoning Process : **

The child exhibits several symptoms : a short attention span , distractibility , forgetfulness , hyperactivity , and difficulties with listening . These symptoms are characteristic of Attention - Deficit/Hyperactivity Disorder (ADHD), which is a neurodevelopmental disorder .

1 . ** Short attention span , distractibility , and forgetfulness : ** These symptoms suggest difficulties with executive functioning , particularly in tasks requiring sustained attention and working memory . These are hallmark features of ADHD , which often involves challenges with executive functioning .

2 . ** Hyperactivity and inability to concentrate on tasks longer than a few minutes : ** These are classic symptoms of hyperactivity and impulsivity , which are core diagnostic criteria for ADHD . The child 's inability to concentrate and hyperactivity can lead to difficulties in academic and social settings .

3 . ** Intermittent episodes of acute otitis media : ** While the child has had ear infections in the past , the current presentation does not strongly suggest that these are directly related to the current symptoms . The history of ear infections could potentially suggest a hearing issue , but the child appears healthy and well-nourished , and there are no other specific findings on examination to suggest hearing impairment .

4 . ** Family history : ** The mother 's major depressive disorder and the father 's Graves ' disease are noted but do not appear to have a direct link to the child 's symptoms . While genetic factors can play a role in ADHD , the absence of other symptoms like withdrawal or aggressive behavior makes primary ADHD the most likely diagnosis .

5 . ** Behavioral symptoms : ** The child 's difficulty following instructions , being easy to distract , and interrupting others are consistent with inattention and impulsivity , which are part of the ADHD spectrum .

** Conclusion : **

Based on the symptoms of short attention span , distractibility , forgetfulness , hyperactivity , and difficulties with listening , the most likely diagnosis for the child is Attention - Deficit/Hyperactivity Disorder (ADHD). The appropriate next step in treatment would be behavioral therapy , which is a common and effective intervention for managing ADHD and addressing related behavioral issues .

</think >

<answer >

Behavior therapy

</answer >